



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متدد

إسم المريض رباغياً : امال فايز شفيق صليب
المحافظة : بورسعيد
الحى / المدينة : بورفؤاد
رقم البطاقة : 26907162501023 رقم التليفون : 01273413898
العمر (تاريخ الميلاد) : 16-07-1969 المهنة : ربه منزل
العنوان : =====
تاريخ تسجيل الإستمارة : 01-10-2026

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)	
نتائج الأبحاث : (مرفق)	
التشخيص النهائي : سرطان بالثدي (مبادرة)	
خطة العلاج : أبحاث أورام اشعه مقطعيه علي البطن والصدر والحوض	قسم العلاج : الأورام
بالصبغه [11127]	
التعليق على التشخيص : -----	

خطاب إداري
هذا الرقم القومي يتبع منظومة التأمين الصحي الشامل جهة الاختصاص



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Name: أمال فايز شفيق صليبي National ID: 26907162501023	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced CT Chest & abdomen and pelvis
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٠٢٤/١٢ ٢٦٩٠٧١٦٢٥٠١٠٢٣

ربة منزل

انثى مسيحية متزوجة
سمير حنا عبد الشهيد برسوم

البطاقة سارية حتى ٢٠٣١/١٢/٢٠



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Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
الدارة الإشعاعية

Patient Name: Amaal Fayez Shafik Saib	Date : 22/09/2022
Exam : MAMMOGRAPHY BOTH	Age : 53 Year(s)
Medical ID :	55551

**RIGHT BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Known case of left MRM

Technique

- Right MLO and CC views
- Right 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

RIGHT BREAST:

- Scattered fibro glandular density (ACR B)
- No spiculated dense masses or suspicious microcalcifications

Complementary ultrasound examination revealed no solid or cystic lesions seen.

Normal skin thickness and contour.

Normal nipple/areolar complexes.

- Nonspecific axillary LN.

LEFT OPERATIVE BED:

- No suspicious solid or cystic lesions seen.
- No pathological axillary or supraclavicular lymph nodes.

Opinion:

- Right normal study (BIRADS1)
- Left side no evidence or recurrence

Yours truly

Dr/Asmaa Zeinhom, Msc

Signature Temp

22/09/2022 1:00 PM

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مستشفى أورام دار السلام
الادارة الاشعة

Patient Name: Amaal Fayed Shafik Salib	Date : 09/10/2023
Exam : MAMMOGRAPHY BOTH	Age : 54 Year(s)
Medical ID: 439/2019	Pat. ID: 20221

DIGITAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY

ULTRASOUND

Report:

- Left MRM:
 - Operative bed shows no mass or collection
 - No regional lymphadenopathy.
- Right Breast:
 - ACR B
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Post left MRM status showing no evidence of recurrence (BIRADS 2)
- Normal sonomammography of the right breast (BIRADS 1)

Signature Dr. Peter Nashat

09/10/2023 2:27 PM

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مادة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
الدارة الاشعة

Patient Name: Amaal Fayed Shafik Salib	Date : 22/09/2022
Exam : C.T CHEST WITH CONTRAST	Age : 53 Year(s)
Medical ID:	

POST CONTRAST MULTI-SLICE CT SCAN OF THE

CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Evidence of left side mastectomy with clear operative bed.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- No destructive bony lesions



ERM036746338

جمهورية مصر العربية
بطاقة تحقيق الشخصية

افعال
فايز شفيق صليب

١٩ ش محمد عبدالسيد- الدكتور فؤاد السيد
العمرانية - الجيزة

١٩٦٩/٠٧/٢٦

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HG6066269



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٢٦٩.٧١٦٢٥.١.٢٣

رية منزل

متروجة

مسيحية

أنثى

سمير حنا عبد الشهيد برسوم

البطاقة سارية حتى ٢٠٢٤/١٢/٠٦

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Pathology Report

Name: امال فايز شفيق

Age: 50 years.

Clinical data: Left breast mass.

Specimen Type: True cut biopsy.

Date: 19/2/2019

Lab. Number: 111 **Hospital Number:** 239/2019

Gross Description:

Three greyish-tan tissue cores up to 0.6 cm. Totally submitted.

Microscopic Examination:

Histopathological examination of sections prepared reveals an infiltrating malignant epithelial growth. The tumour is made up of groups and small sheets of markedly pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is marked. Cellular and nuclear atypia are marked [grade III].

Diagnosis:

Left breast mass, true cut biopsy, invasive duct carcinoma, GIII.


Prof. Dr. Amr ElSebai

Dr. Marwa Mahmoud



Ministry of health
Central health laboratories
Pathology department

ImmunohistoChemistry report

Report Number: m1620

Female	: نوع	أمال فايز شفيق	: اسم المريض
القاهرة	: العنوان	50 Y	: السن
13/03/2019 12:17	: تاريخ استلام العينة		: تاريخ العينة
24/03/2019 13:11	: تاريخ التقرير	مركز اورام دار السلام	: النمل
		مركز اورام دار السلام	:

Primary Sample :

Paraffin Block

Test :

ER & PR & HER2 neu (ER & R² Harvey scoring system - Her2neu : CAP scoring system)

Comment :

Technique :

sample recieved was treated by: - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti progesterone receptor (PR) (1E2) Rabbit monoclonal primary antibody.

Sample recieved was treated by: - VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX.

Result :

ER: NO nuclear staining of tumour cells

PR: NO nuclear staining of tumour cells

HER2/neu protein over expression : discontinuous membrane staining of tumor cells.

Referral Lab :

Comment :

Conclusion :

ER: NEGATIVE

PR: NEGATIVE

HER2/NEU PROTEIN OVER EXPRESSION : NEGATIVE (SCORE 1+)

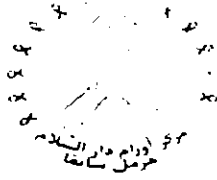
Pathologist:

Lab Director: د/ تريزا ماهر

19 El-sheikh Rehan Street, Abe.deen, Cairo
Tel.(165)7948544 -7950096 Fax. (202)7948544
Mobile . 01144297292
website: <http://www.healthlabs.net>
email: pathologychl@yahoo.com
facebook: [صم البقاولجر - الإدارة المركزية للمعامل](#)

This Test Is Not Accredited By EGAC Nor Recognized By

يتم الاحتفاظ بأصل العينة لمدة 30 يوم من تاريخ استلامها



Patient name: «مال فايز شفيق»

Date: 12/12/2021

BILATERAL HIGH RESOLUTION ULTRASOUND

Indication:

Patient had left MRM

Her last SM was on 3/20221

Report:

- Homogenous back ground echotexture
- no right suspicious mass , cyst.
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Right Nonspecific axillary LN, with preserved fatty hila and uniform cortical thickness .
- Free left operative bed with no evidence of recurrent mass lesions.
- Free left infra and supraclavicular lymph node .
- NB: right congenitally retracted nipple.

Opinion:

- left MRM with free operative bed with no evidence of recurrent mass lesions.
- Right breast normal study.(BIRADS 1).

Thanks for referral

dr Engy Abdullah



Patient name: امال فليز شفيق

Date: 10/3/2021

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Patient had left MRM

Technique

- right MLO and CC views
- Bilateral ultrasound

Report:

- The right breast shows scattered fibro glandular density (ACR B).
- No right breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Complementary ultrasound shows no right suspicious mass , cyst , or microcalcification .
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Right Nonspecific axillary LN, with preserved fatty hila and uniform cortical thickness .
- Free left operative bed with no evidence of recurrent mass lesions.
- Free left infra and supraclavicular lymph node .
- NB: right congenitally retracted nipple.

Opinion:

- left MRM with free operative bed with no evidence of recurrent mass lesions.
- Right breast normal study.(BI-RADS 1).

Thanks for referral

dr nehal hamdey, FEBR. MSC

Pathology Report

Name: أمال فايز شفيق

Age: 50 years.

Clinical data: Left breast mass.

Specimen Type: Left modified radical mastectomy.

Date: 7/3/2019

Lab. Number: 165

Hospital Number: 439/2019

Gross Description:

Left modified radical mastectomy, breast tissue; 23x13x2.5 cm. Skin ellipse; 16x8 cm including nipple and areola. Breast showed ill-defined firm greyish white mass, 3x2 cm with least deep margin 0.2 cm. Axillary tissue measuring 11x9 cm. Dissection revealed 15 nodules. Largest was 1.5x1 cm.

Microscopic Examination:

Histopathological examination of sections prepared from the tumour mass reveals an infiltrating malignant epithelial growth made up of groups and sheets of pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is moderate. Cellular and nuclear atypia are moderate [grade II]. No intraductal component could be detected. All of the 15 lymph nodes detected show only reactive hyperplasia [Nodal score is 0/15]. The nipple, areola and surgical margins are free. Surrounding breast tissue shows fibrocystic changes.

Diagnosis:

Left modified radical mastectomy, widely necrotic Invasive duct carcinoma, GII.

- Positive lymphovascular invasion.
- Free margins, nipple, areola.
- Surrounding breast tissues show fibrocystic disease.
- Free lymph nodes: (0/15).
- Stage: T2N0Mx.

Prof. Dr. Amr ElSebai

Dr. Marwa Mahmoud

Prof. Dr. Essam Ayad

PATIENT NAME: AMAAL FAYEZ SHAFEK SALIP	PATIENT ID #: 2018069088
GENDER: Female	ORDER #: 336809
AGE: 49 y	DATE OF SERVICE: 25-Dec-2018 04:31 PM

Report:

Full Field Digital Mammography

TECHNIQUE:

- CC and MLO view.

REPORT:

- ACR C: Heterogenously dense fibro-glandular parenchyma ,lowering the sensitivity of the study.
- Left breast upper central, partially obscured focal asymmetric density is noted.
- No right speculated mass lesions or areas of architectural distortion.
- No suspicious pleomorphic micro-calcific clusters.
- Normal skin thickness and contour.
- Normal nipple /areola complexes.

Opinion:

- Left breast upper central, focal asymmetric density (BIRADS 0), warranting further sonographic assessment.

Reported By,

Dr. Peter Fahmy, MSc

25-Dec-2018 04:13 PM